جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : جملات مصطفى محمد السيد
المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** الجيزه
رقم البطاقة : 26812141202327 **رقم التليفون :** 01235684589 **تاريخ تسجيل الإستمارة :** 21-10-2026
العمر(تاريخ الميلاد) : 14-12-1968 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: جملات مصطفى محمد السيد
National ID: 26812141202327

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

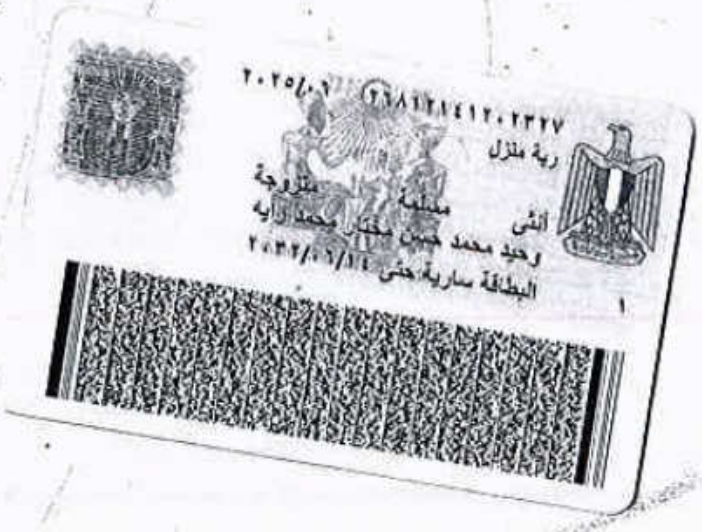
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bilateral mammogram and breast ultrasound**

التاريخ : **05-07-2026**	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

AL SHROOUK SCAN
الشروق بلازا

15184

Pat. Name: Gamalat Moustafa Mohamed Al Sayed	Pat. ID: 20654601
Pat. Age 57 Year(s)	Pat. Sex F
Study ID:	Study End Date: 05/01/2026 17:29:31
Procedure Name: Complementary echocardiogram	Ref. Phy. -- --

ECHOCARDIOGRAPHY

M.MODE MEASUREMENTS

LVEDD	5.17	N (3.5-5.6 cm)	Aorta	2.52	N (Up to3.7cm)
LVESD	3.57		LA	4.04	N(Up to 4cm)
IVS	0.83	N(0.6-1.1 cm)	RV		N(.7-2.3 cm)
PW	0.74	N(0.6-1.1 cm)	FS	31%	
			EF	58%	N (=50%)

COMMENT:

- Normal left ventricular internal dimensions with Good systolic function .
- No RSWMA at rest.
- Normal LV filling pattern .
- Upper normal LA internal dimension .
- Mild mitral valve regurge .
- Normal aortic valve with good excursion, no gradient. Normal flow.
- Mild to moderate tricuspid valve regurge with PG 29 mmHg .
- Normal pulmonary valve thickness and excursion. No regurgitation
- Normal right ventricle dimension and contractility.
- No pericardial effusion.
- No intracardiac masses or thrombi by TTE at time of study.

Conclusion:

- *Mild MR , Mild to moderate TR .*

Dr. Ahmed Anwar
MD, Cardiology

Name: جملات مصطفي محمد السيد	
MR. : 6739	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient received Cycle 4 of Herceptin subcutaneous (SC) with adjustment plus Femara; an echocardiogram showed an ejection fraction of 58 %, and a breast ultrasound on 5 January demonstrated a postoperative mastectomy bed seroma. The patient subsequently proceeded to Cycle 6 of Herceptin SC as documented by pharmacy.

During a clinical pharmacy visit, the patient was noted to be on Cycle 5 of Herceptin SC with Femara, with an echocardiogram on 7 April 2026 showing an ejection fraction of 65 %. The plan included Femara 1 mg daily and supportive medications for Cycles 6 and 7 as ordered by pharmacy.

A pharmacy intervention clarified that the patient was prescribed Arimidex rather than Femara.

The pharmacy record indicated that the patient received Cycle 6 of Herceptin SC together with Arimidex.

The patient was administered Cycle 8 of Herceptin SC; the mastectomy bed seroma persisted with traumatic fat necrosis noted, and the patient continued Arimidex. The patient has completed eight cycles of Herceptin subcutaneous.

تحريرا في ,

التاريخ :	19-09-2026	الوقت :	

الطبيب المعالج : هنا اسامه عبد الرحيم عبد البر

يعتمد ,

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصرية هي صحة مصر

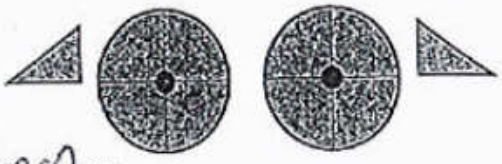
MDT FORM 4/11/2025 G

Name	محاسن مصطفى محمد	MRN	1977/6 c0
		ID	CTAIC1212C.C2 CV
Age	56Y.	Phone	01147409903
Menopausal status	Postmenopausal	Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity	DM, HTN		

Clinical Data

BX: IDC II

R ps:1 L



ER: +ve 8/8	PR: +ve 6/8	HER 2: 2+	Ki67: 30%

Sonomammography: CEDM: Amplified posttx D lefr Breast

Mets Workup: Locally adv Rec MBCT at 3 AC, 3 Taxotere

CT4, N2, M0 TNM

Recommendations: on 9/8/2025 :- lefr MRM. YPT2 14PN1 1/3. for RDT Irradiation

MDT OUTCOME & PLAN

① Adjuvant Herceptin + AI + Zometa / 6 months.

② RDRT

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

معامل الخليـــة
لتحاليل الأنسجة والأورام

Patient Name	جلالات مصطفى محمد السيد	Code Number	YK 25024073
Sex/Age	Female 57 Y 0 M 0 D	Patient ID	62782
Registered Date	30/7/2025 11:10:21 PM	Referred By Dr.	محمد صلاح
Validation Date	06/8/2025 09:51:57 PM	Branch	الفرع الرئيسي الدقي
Reporting Date	09/8/2025 10:16:13 AM		

Clinical Diagnosis: Left Breast Mass
Nature of the specimen: Left MRM, Axillary Lymph Nodes And Apical Lymph Nodes

Pathology Report

Gross Picture

Two designated specimens were received as:
A) MRM specimen measured 22x20x8 cm covered by skin ellipse measured 27x21 cm with attached axilla measured 15x10 cm including nipple and areola. Serialling revealed an ill-defined firm greyish white mass measured 4.3x3.8x2.5 cm, locating 1/2 cm from deep margin and 1.5 cm from the skin. The dissection of axilla revealed (13) nodules measured up to 3x2.5 cm.
B) Apical lymph node: Multiple irregular fibrofatty tissue pieces collectively measured 2.5x2.5 cm entangling (11) nodules measured up to 1x1 cm.

Microscopic Picture

(A) Sections examined from breast masse revealed malignant neoplastic growth formed of tubules and sheets of malignant ductal cells with moderate degree of pleomorphism with surrounding desmoplastic stroma. No Ductal insitu component seen. Lymphovascular emboli seen. No evident Perineural invasion. Sections examined from nipple, areola and all surgical margins were free. One out of 13 dissected axillary lymph nodes was positive for tumor deposits (1/13) without extra capsular extension.
(B) Apical lymph nodes: Sections examined from all dissected lymph nodes were free. (0/11)

Diagnosis

Left Breast Mass, Left MRM, Axillary Lymph Nodes And Apical Lymph Nodes:
A) Infiltrating Duct Carcinoma, Grade II.
No Intraductal Components.
No Perineural Invasion
Lymphovascular Emboli Present
Free Nipple And Areola.
Free Surgical Margins
Axillary Lymph Nodes: Positive (1/13), No Capsular Infiltration.
B) Apical Lymph Nodes: Negative (0/11).
Tumor Stage: pT2N1A.

Scan Me

الفرع الرئيسي الدقي : ٢٠ شارع التحرير أمام مخبز البحوث - الدقي - الدور الأول
م ٠١١١٤٤٣٣٣٣ - ٠١١١٥٥٧١١٧ - ت: ٣٣٣٦٥٣٠٧٣

معامل الخليـــة
لتحاليل الأنسجة والأورام

Patient Name	وسلات مصطفى محمد السيد	Code Number	YK3502673
Sex/Age	Female 57 Y 0 M 0 D	Patient ID	6781
Registered Date	30/7/2025 11:10:31 PM	Referred By Dr.	محمد صلاح
Validation Date	05/8/2025 09:51:57 PM	Branch	فرع فرنسي فار
Reporting Date	03/8/2025 10:16:13 AM		

P.O

Number of slides enclosed: 9

Diagnosed by

Consultant Pathologist
Dr. Ahmed Nasser, MD



Scan Me

شارع التحرير أمام مخرج الجيزة - الدقي - الجيزة

Histopathology Report

Patient Name	: الأستاذه / جملات مصطفى محمد السيد	Request No.	: 13002786
Patient No.	: 26812141202327	Request Date	: 24/06/2025
Path No.	: SR-857/25	Verification Date	: 29/06/2025
Sex	: Female	Financial Category	: اقصادى -
Age	: 56 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data:
A case of left breast mass with axillary lymph node was diagnosed elsewhere as invasive duct carcinoma with lymph node macrometastais (Her2 result was equivocal in one lab & was positive in other lab).

Nature of Specimen:
Referred blocks & slides (Tru-cut biopsies), Requested SISH.

Gross Examination:
- Two referred blocks labelled as (25010524). Together with seven referred slides including 4 immunostained slides for (ER, PR, HER2, Ki 67) and three H&E stained slides sublabelled as (1, 2 & 5).

Technique:
- Sections prepared from paraffin blocks for immune-staining using Bench Mark ULTRA system for:
- SISH.

* IPT:
- SISH: Amplified.
'Average Her2 copy No.: (7.85), Her2/CEP17 ratio: (2.8)).

Diagnosis:
Left breast mass with axillary lymph node, Referred blocks (Tru-cut biopsies), Requested SISH):
SISH: Amplified, Group I.
Average Her2 copy No.: (7.85), Her2/CEP17 ratio: (2.8)).

SH groups:

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Histopathology Report

الأستاذه / جملات مصطفى محمد السيد :		Request No.	: 13002786
	: 26812141202327	Request Date	: 24/06/2025
	: SR-857/25	Verification Date	: 29/06/2025
	: Female	Financial Category	: اقصادى -
	: 56 Y	Department	: Out Patient
ing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

e ratio is calculated and the results is interpreted as:
Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
• Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
• Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
• Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
• Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Prof. Dr. Maha Guimei.
Dr. Duaa Mohamed.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name:	Jamalat Mustafa Mohammed	Patient ID:	593029
Prof. Dr:	Mohammed Salah Hassan	Exam Date:	29/4/2025
Accession:	250427336		

Whole Body PET/CT Examination

History and Clinical Data:-

Patient data:-

Female patient, 57 years old, Weight: 132 Kg, Height: 172 cm, Blood Glucose level: 84 mg/dl

Radiopharmaceutical:-

F-18-FDG 11 mCi injected I.V.

Clinical background:-

- A case of left breast cancer.

Purpose of the study:-

- Initial assessment.

Technique:-

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the knees to the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~2.3.
- Mediastinal blood pool average SUV ~1.59.

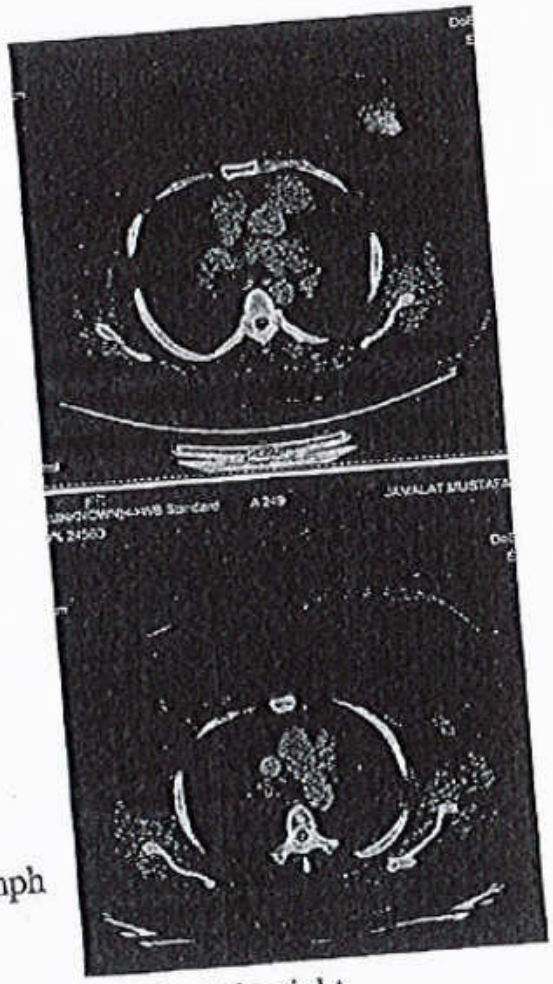


Patient Name:	Jamalat Mustafa Mohammed	**Patient ID:**	593029
Prof. Dr:	Mohammed Salah Hassan	**Exam Date:**	29/4/2025
Accession:	250427336		

Findings:-

I) Breasts and Axillary regions:-

- Metabolically active left breast soft tissue mass is noted at its lower outer quadrant measuring 4.2 x 4.7 x 4.4 cm.
- Associated with overlying metabolically inactive (non FDG avid) skin thickening up to 1 cm with max SUV ~ 6.45.
- Metabolically active few enlarged left axillary lymph nodes are noted measuring 2.8 cm with max SUV ~ 3.50.
- No metabolically active right breast focal lesions
- No metabolically active right axillary lymph nodes
- Metabolically inactive (non FDG avid) few subcentimetric right axillary lymph nodes (likely inflammatory)



II) Head and Neck:-

- Scanned brain sections show uniform bio-tracer distribution in the cerebral and cerebellar hemispheres.
- No metabolically active (FDG avid) masses are noted.

CT Scan
[[DOCTYPE CT_SCAN]]

Patient Name:	Jamalat Mustafa Mohammed	Patient ID:	593029
Prof. Dr:	Mohammed Salah Hassan	Exam Date:	29/4/2025
Accession:	250427336		

- No metabolically active (FDG avid) cervical lymph-nodes are noted.
- Metabolically inactive (non FDG avid) few subcentimetric bilateral cervical level II lymph nodes; likely inflammatory
- Bilateral maxillary sinusitis.

III) Chest:-

- No metabolically active (FDG avid) pulmonary nodules or masses are noted.
- Metabolically inactive (non FDG avid) middle lobe thick atelectatic bands (likely post inflammatory).
- No metabolically active (FDG avid) pleural effusion is noted.
- No FDG avid (metabolically active) mediastinal lymph-nodes are noted.

IV) Abdomen and pelvis:-

- Average sized liver showing no metabolically active (FDG avid) focal lesions.
- Average sized spleen showing no metabolically active (FDG avid) focal lesions.
- No metabolically active (FDG avid) porta hepatis, mesenteric, retroperitoneal or pelvic lymph-nodes are noted.
- No ascites.
- Left renal non-obstructive lower calyceal stone.

Patient Name:	Jamalat Mustafa Mohammed	**Patient ID:**	593029
Prof. Dr:	Mohammed Salah Hassan	**Exam Date:**	29/4/2025
Accession:	250427336		

- The rest of the abdominal and pelvic organs show normal PET/CT appearance with no related focal lesions or abnormal FDG uptake.

V) Axial and scanned Appendicular Skeleton:-

- No (FDG avid) metabolically active osteolytic or osteosclerotic lesions are noted.

Conclusion:

A known case of left breast cancer showing:

- ***Metabolically active left breast neoplastic mass as described.***
- ***Metabolically active few metastatic left axillary lymph nodes.***
- ***No metabolically active distant metastatic deposits.***

Thank You for Your reference

PET/CT Unit

Dr. Beshoy Nabil, MSc
Revised by Dr / am
Typed by, Youssef Hazem

Radiology consultant
Prof. Dr. Ramy Edward, M.D